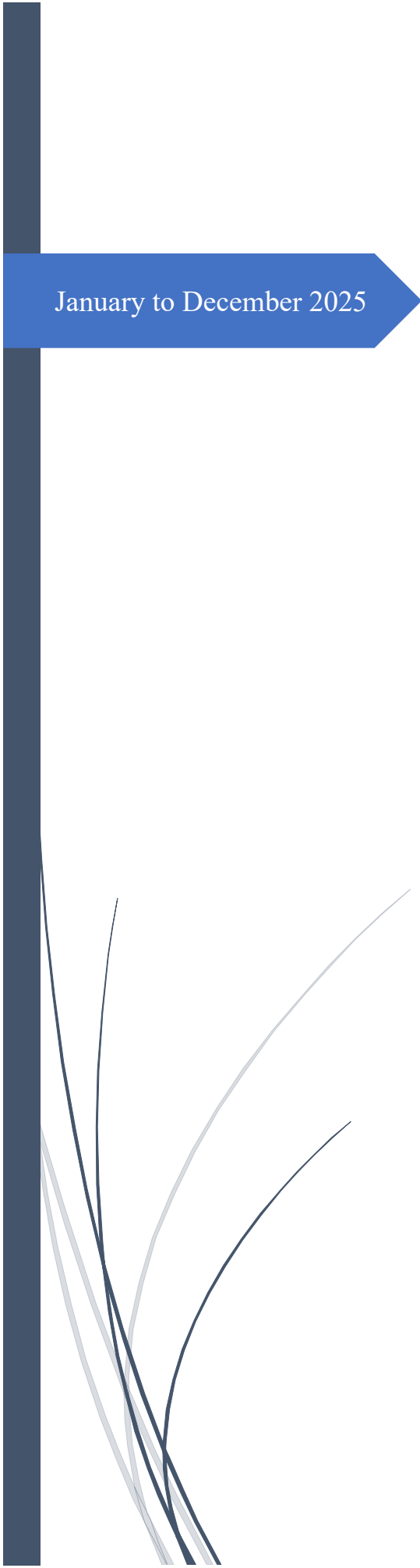




January to December 2025

ANNUAL STI DATABASE MONITORING REPORT

HIV/STI/TB UNIT
STRATEGIC INFORMATION COMPONENT

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Executive Summary

The STI database compiles monthly reports from the four Regional Health Authorities across Jamaica, with reporting volumes determined by the number of health centres providing STI testing and treatment services within each region. As at the end of December, a total of 1,104 reports were expected nationally, comprising 216 from the Western Region, 156 from the North East Region, 324 from the Southern Region, and 408 from the South East Region. From January to December 2025, national reporting completeness remained high across most regions; however, a notable decline was observed beginning in October following the catastrophic impact of Hurricane Melissa.

The category five hurricane struck the island in October 2025 causing damage to health facilities, power outage and interruption in internet connectivity. This affected real time data entry and access to the DHIS2 database especially among the most affected parishes in the Western Regional and Southern Health Authorities. Displacement of health centre workers and patients in the affected parishes further compounded the routine updating of the database and health seeking behaviours of the target populations which may have disrupted the timeliness and completeness of reports during the period of October to December 2025. Despite generally strong completeness, timeliness remained a persistent challenge throughout the year, particularly in the South East Region, where on-time submission rates consistently remained below 10%.

Timely and comprehensive reporting is fundamental to the integrity of national surveillance data and supports informed, evidence-based decision-making and policy formulation for the effective management of sexually transmitted infections. The database monitors trends and occurrences of STIs using key indicators such as cases by sex and age group, genital ulcer disease, genital discharge syndrome, syphilis, epidemiological treatment of sexual partners for chlamydia and gonorrhea, ophthalmia neonatorum (in infants under one month of age), epididymo-orchitis, pelvic inflammatory disease (PID), genital warts, pediculosis, scabies, other STIs, and HIV testing.

As at December 2025, a total of 36,836 sexually transmitted infection (STI) cases were reported nationally. Females accounted for the majority of cases, representing 77.9% (28,709), while males comprised 22.1% (8,127). The greatest burden of infection occurred among individuals aged 20–29 and 30–39 years, followed by those aged 50 years and older. Genital discharge remained the most frequently reported syndrome during the year, with genital herpes and syphilis at comparatively lower levels. Among Other STIs and Non-STI conditions, Pelvic Inflammatory Disease, Genital Warts, and epidemiological treatment for gonorrhoea/chlamydia were most commonly reported. In contrast, epidemiological treatment of partners for syphilis and cases of epididymo-orchitis were reported less frequently. Despite females constituting the majority of clinic attendees, HIV and syphilis testing coverage among females was substantially lower than that observed among males.

These indicators support ongoing monitoring of disease trends, assess the effectiveness of prevention and treatment interventions, and enable the identification of high-burden populations for targeted action. Strengthening data quality, completeness, and timeliness remains essential to sustaining a robust and reliable STI surveillance system.

Reporting Compliance

Throughout the year, all regions showed strong reporting completeness early on, followed by a noticeable mid to late year decline, with varying degrees of recovery toward December (Figure 1.1). The Western Region maintained perfect (100%) reporting from January through July, a slight dip in August (94%), followed by a brief recovery in September (100%), then declined sharply in October due to the severe impact of Hurricane Melissa which made landfall in the west side of the island, before stabilizing at a lower but consistent level through November and December (83%). The North East Region started the year strong but experienced a gradual and sustained decline from mid-year, falling from consistently high performance in the first half with 100% completeness in most months to its lowest levels by November and December (69%), with no significant rebound. The Southern Region showed relatively stable and high completeness reporting in the first half with 93% in most months, a dip around July (74%) and August (78%), a brief recovery in September (95%), followed by a decline in October (67%) as St. Elizabeth was also devastatingly damaged by the hurricane and partial stabilization (81%) toward year-end. The South East Region demonstrated the most pronounced deterioration, dropping sharply after June to very low levels in July (7%) and August (10%), with only modest improvements thereafter before further decline in December (9%), ending the year well below early-year performance.

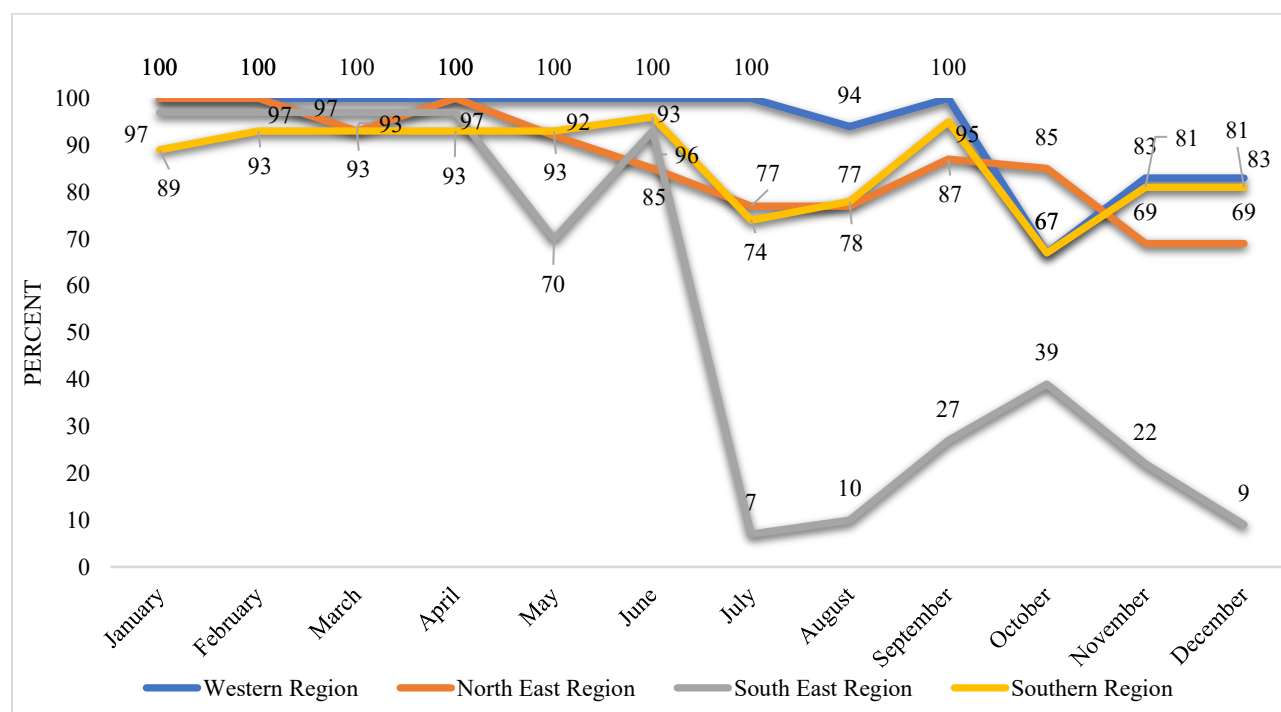


Figure 1.1 Completeness of Submitted Reports by Region

From January to December, the timeliness of submitted reports shows clear regional differences. Most regions experienced improvements in timeliness of reporting by mid-year followed by a sharp decline in the final quarter, with uneven recovery by December. The Western Region showed steady improvement from January, peaking at perfect timeliness (100%) in June and July, remaining consistent through September, before a significant drop in October (22%) and November (50%) as they begun recovery from Hurricane Melissa, followed by a significant rebound in December (83%). The North East Region experienced weak and inconsistent performance early in the year, improved gradually from April (46%) to September (80%), then declined again in October (46%), with modest recovery by year-end (69%). The Southern Region demonstrated gradual strengthening from January (63%) to June (85%), sustained relatively good performance through September (94%), followed by a sharp drop in October (33%) due to the hurricane and notable recovery by December (81%). The South East Region consistently recorded the lowest timeliness throughout the year, with minimal improvement at its' mid-year peak (47%), indicating persistent challenges regarding timeliness of reporting across all periods (Figure 1.2).

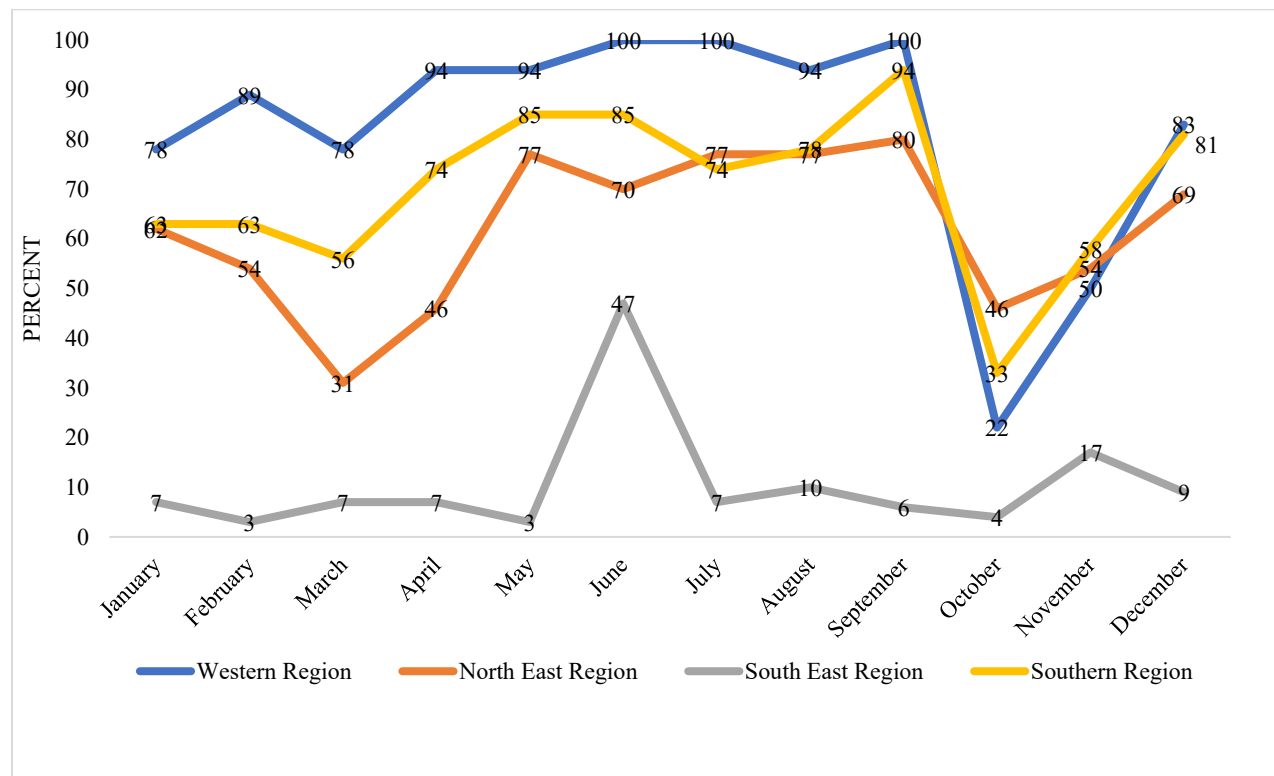


Figure 1.2 Timeliness of Submitted Reports by Region

STI Surveillance

Gender

A total of 36,836 STI cases were recorded as at December 2025, with 77.9% occurring among females (28,709) compared to 22.1% males (8,127). Throughout the year, reported STI cases showed a general downward trend for both males and females, with higher volumes recorded in the first half of the year and a steady decline from July through December. Females consistently represented the majority of reported cases throughout the year, accounting for between 70.7% and 80.4% of infections. In contrast, males contributed a smaller share, ranging from 19.6% to 29.3%, indicating a gender disparity in reported infections. Total cases peaked January to June, followed by notable reductions in August and September, reflecting the lowest reporting levels of the year. Although overall case numbers remained lower in the October to December quarter, the proportion of male cases reported increased notably in October (29.3%) and November (25.5%), suggesting improved uptake by males despite reduced totals (Figure 2.1).

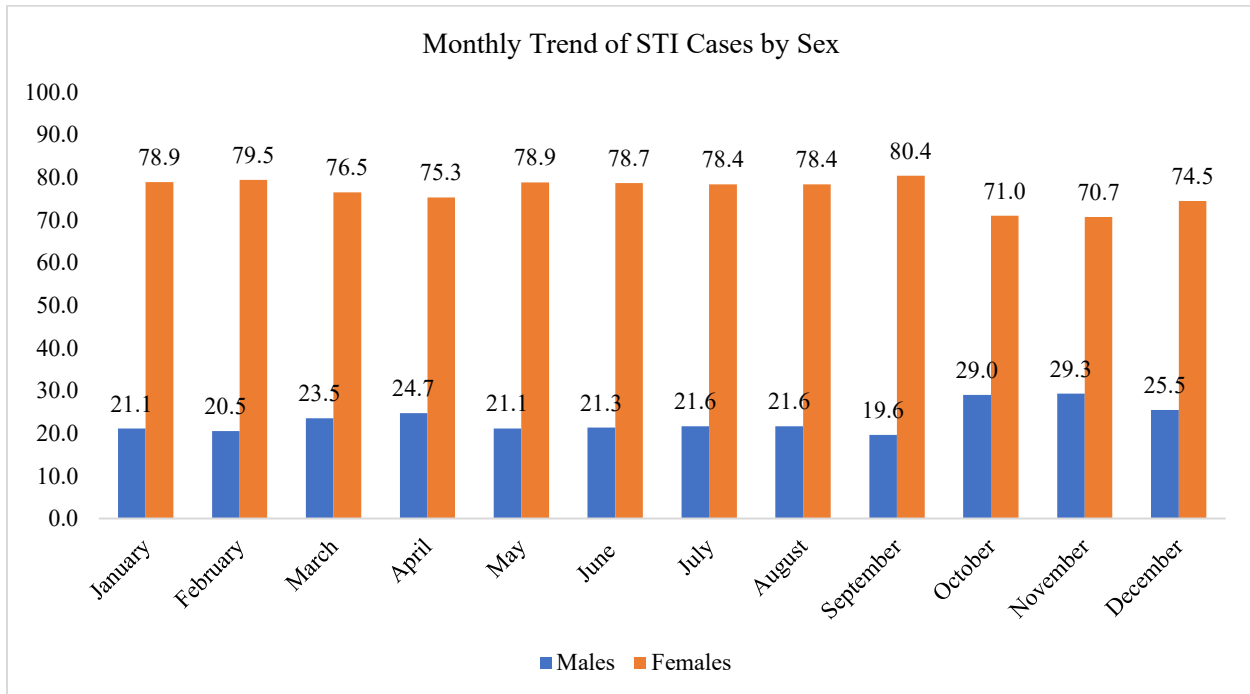


Figure 2.1 Monthly Trend of STI Cases by Gender

Age

STI cases throughout the year were concentrated primarily among adults aged 20–29, who consistently recorded the highest number of cases each month (28%-32%), followed by those aged 30–39 (21%-24%) and 50+ (15%-19%). A notable decline was observed across most age groups from mid-year onward, with steady reductions between July and September before a resurgence in October and November, particularly among individuals aged 20–29 and 30–39. Adolescents aged 15–19 experienced a gradual decline after June, with proportions peaking at 13% and falling to a low of 10%. In contrast, cases among children aged 0–9 and 10–14 remained comparatively low throughout the year but showed some fluctuation, including a notable spike among the 0–9 age group in November (3%). Older adults aged 50+ maintained relatively high case numbers throughout the year, with a peak increase evident in September (19%). Cases recorded as “Unknown” were minimal overall, with an isolated increase in April (3%). The data indicates higher rates of STI transmission in the first half of the year, followed by a general reduction across most age groups toward the end of the reporting period (Figure 2.2).

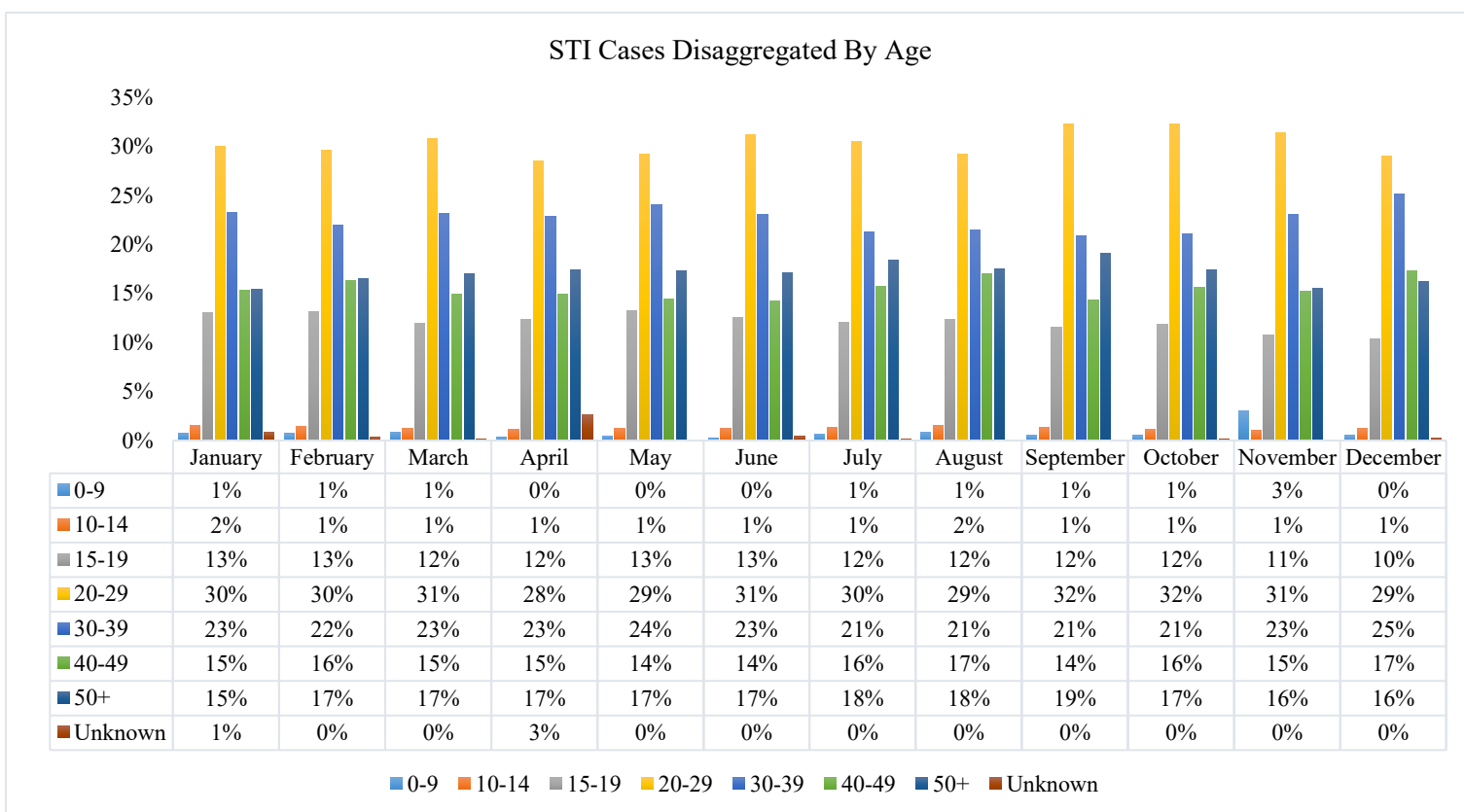


Figure 2.2 STI Cases Age Breakdown

Genital Discharge Syndrome

The STI syndrome distribution across the year shows a consistently high burden of genital discharge syndrome, which accounted for the vast majority of reported cases each month, ranging from 92% to 96% of total syndromic presentations. Levels remained very stable from January through August (94–95%), rose slightly in September (96%), dipped to the lowest point in October (92%), and then rebounded in November (96%) before closing the year at 95% in December. Cervicitis represented a much smaller proportion throughout the year, generally fluctuating between 4% and 6%, indicating a relatively stable but minor contribution to the overall STI syndrome profile. The only notable variation occurred in October, when cervicitis increased to 8%, coinciding with the temporary dip in genital discharge cases, which may be attributable to a reduction in reporting toward the end of the year following Hurricane Melissa and its' possible impact on reduced cases (Figure 2.3).

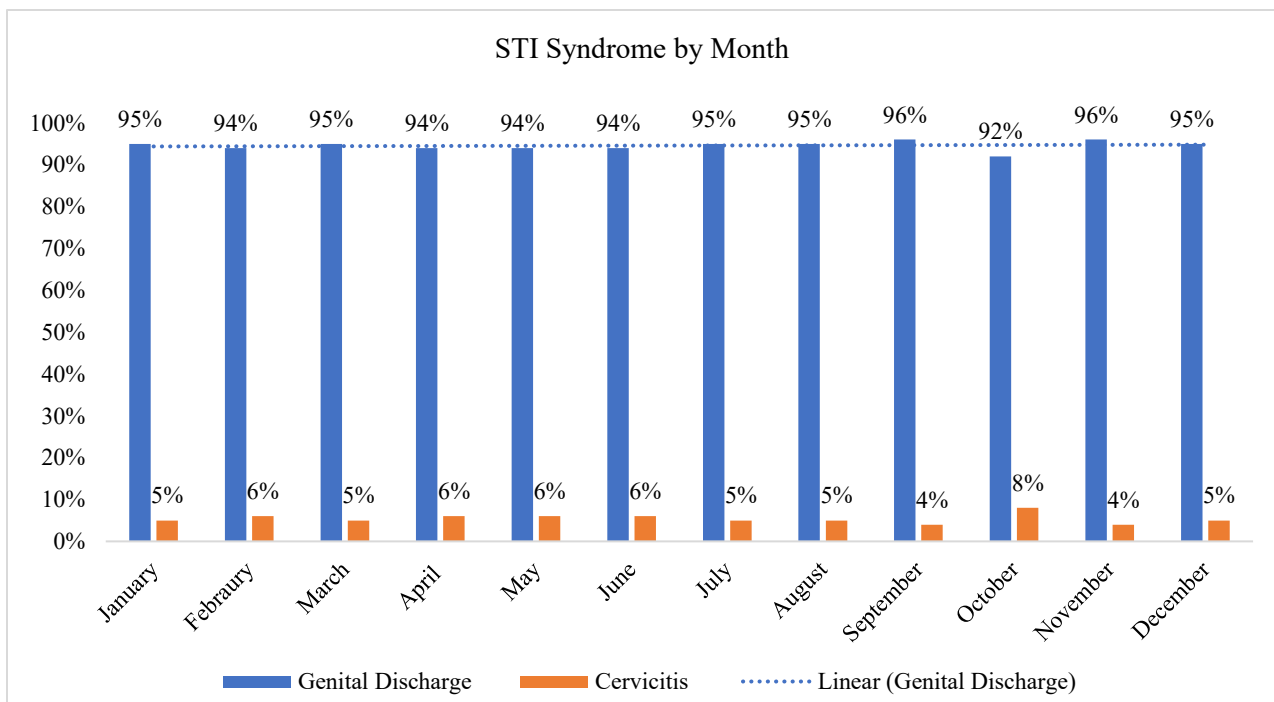


Figure 2.3 Genital Discharge Syndrome by Month

Genital Ulcer Disease

Genital Ulcer Disease (GUD) syndrome patterns over the year exposes genital herpes as the dominant cause throughout all months, though its proportion fluctuated and showed an overall downward trend toward the end of the year which may be due to a reduction in the number of reports received in the last quarter. Genital herpes attained a high of 73% in January, peaked at 85% in February and April, and stayed above 70% for most of the first eight months. However, from September onward, reported cases declined, reaching its lowest level at 55% in December. Contrastingly, syphilis demonstrated a general upward trend with marked mid and late-year increases. After relatively moderate levels early in the year (8–16% from January to May), syphilis rose sharply to 32% in June, remained elevated through the second half of the year, and peaked again at 30% in December. This suggests a growing

contribution of syphilis to the GUD syndrome over time. Cases categorized as “Other” consistently represented the smallest share but showed minor fluctuations, dipping as low as 3% in May and rising to 15% by December (Figure 2.4).

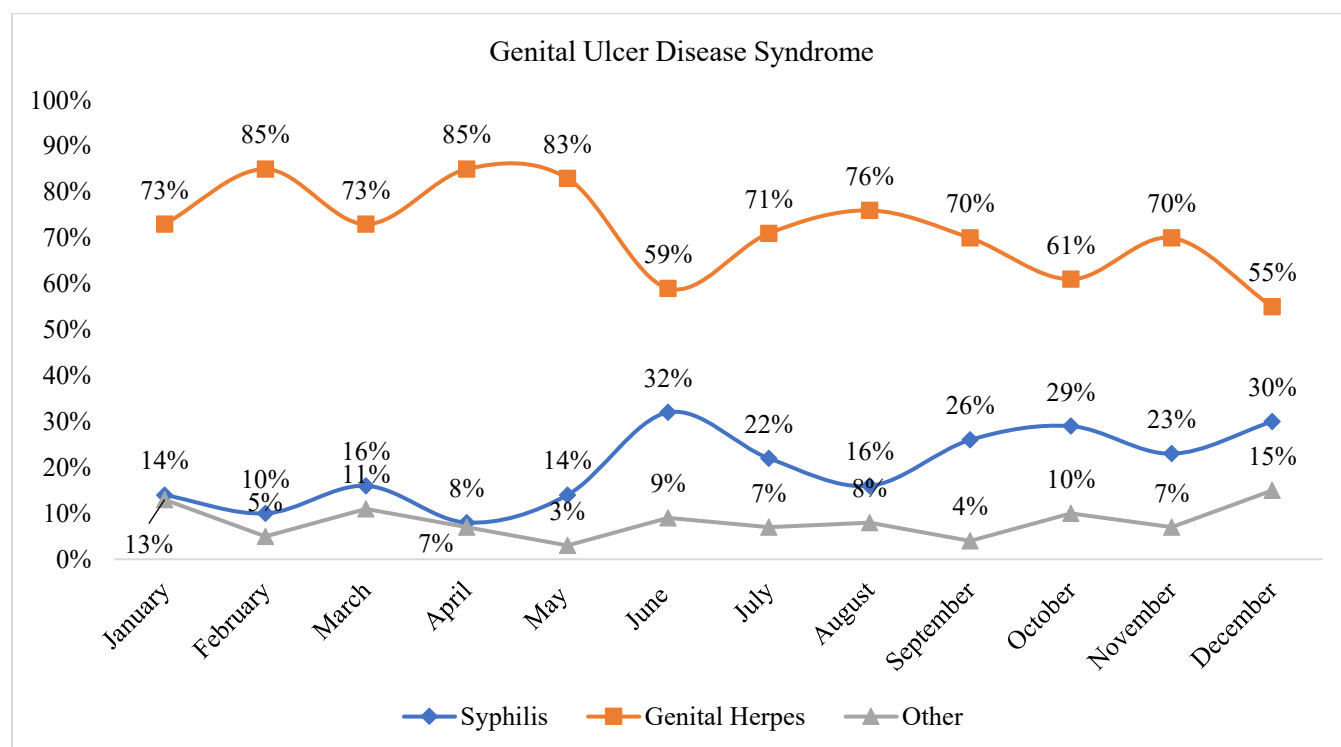


Figure 2.4 Genital Ulcer Disease Syndrome

Syphilis Cases

Syphilis cases were largely driven by latent infections, with Late Latent cases comprising the largest share in most months, peaking notably in September (62%) and remaining high in February and May (58%), before dropping sharply to 24% in November, which may be due to a reduction in reports received in the last quarter. Early Latent infections form the second largest proportion and show an upward trajectory toward year-end, rising from 13% in September to a peak of 36% in December. Secondary syphilis demonstrates marked fluctuations, with moderate levels early in the year before surging in November (34%), the highest value recorded for this stage. Primary syphilis remains comparatively low throughout but shows a small spike in October (16%) and November (13%). There were no reported cases of tertiary syphilis after July and congenital syphilis (<2 years) remain sparse, with isolated cases in April and August (Figure 2.5).

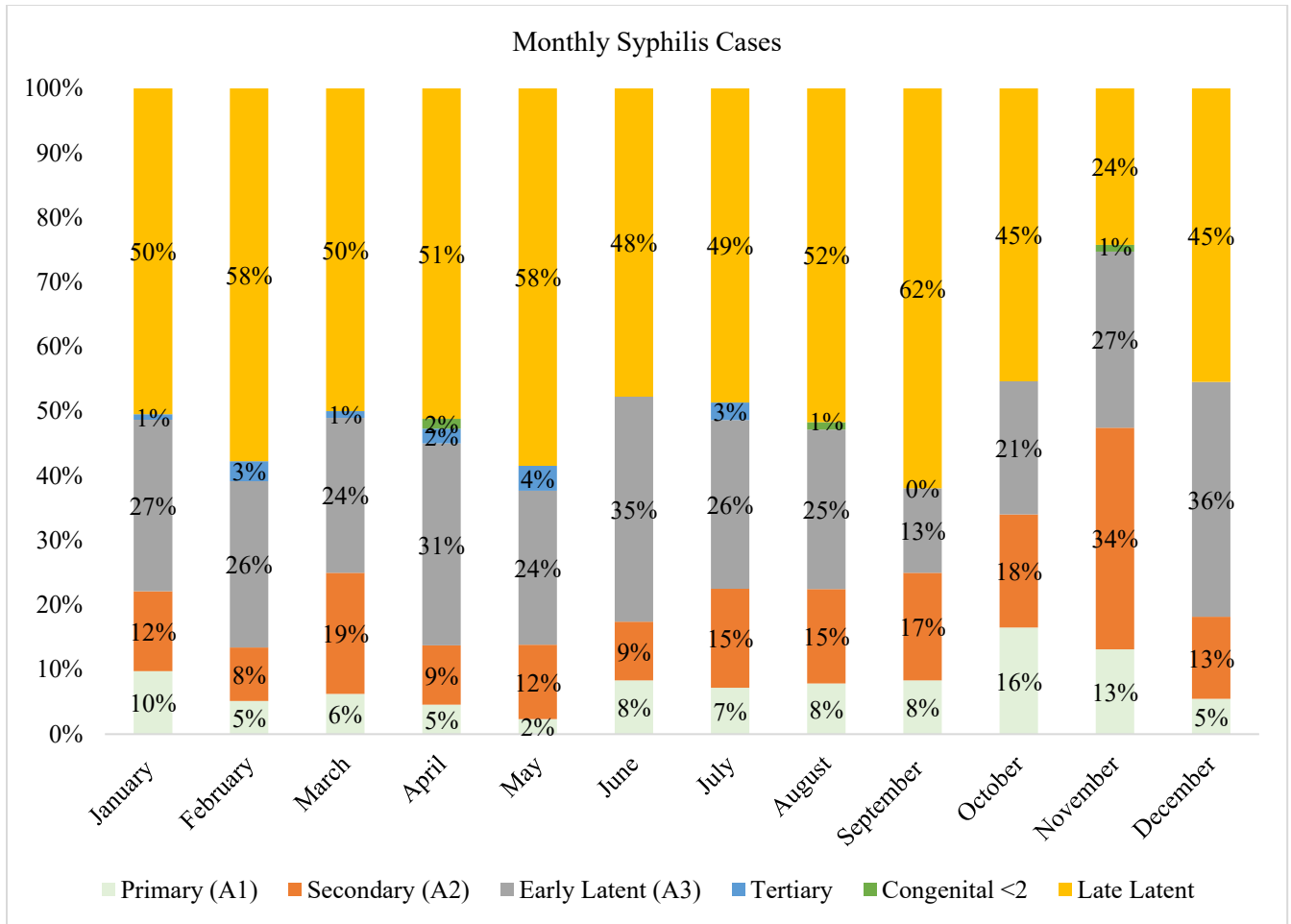


Figure 2.5 Syphilis Cases

Other STIs and Non-STI Conditions

From January to December, other STI and non-STI conditions exhibited fluctuating patterns throughout the year, with several indicators peaking mid-year and again toward year end. Epidemiological treatment for GC/Chlamydia remained consistently high, ranging from 15% to 36%, with notable peaks in September (34%) and November (36%) before declining to 25% in December. In contrast, partner treatment for syphilis remained low and stable overall (1–3%). Pelvic Inflammatory Disease (PID) showed variability rather than a steady decline, starting high in January (29%) and February (27%), rising to a yearly peak in March (36%), then fluctuating through the year before ending at 23% in December. Genital warts displayed noticeable shifts, with high proportions in February (29%) and May (25%), followed by a general decline, reaching its' lowest level in December (5%) despite a temporary spike in October (23%) following Hurricane Melissa.

Cases grouped as “all other STIs” demonstrated the most dramatic variation, increasing from 13% in January and February to a sharp peak in June (42%), before declining in September (18%) and October (13%), and rising again in December (35%). Non-STI conditions such as scabies and epididymo-orchitis remained comparatively low and stable. Scabies ranged between 5% and 11%. Epididymo-orchitis consistently accounted for only 0–3% of cases, peaking briefly in March (3%) and remaining

at 1% in most other months. Ophthalmia neonatorum was sporadic, with 0–4% distribution across the year and a small increase in November (4%), without a sustained upward trend (Figure 2.6).

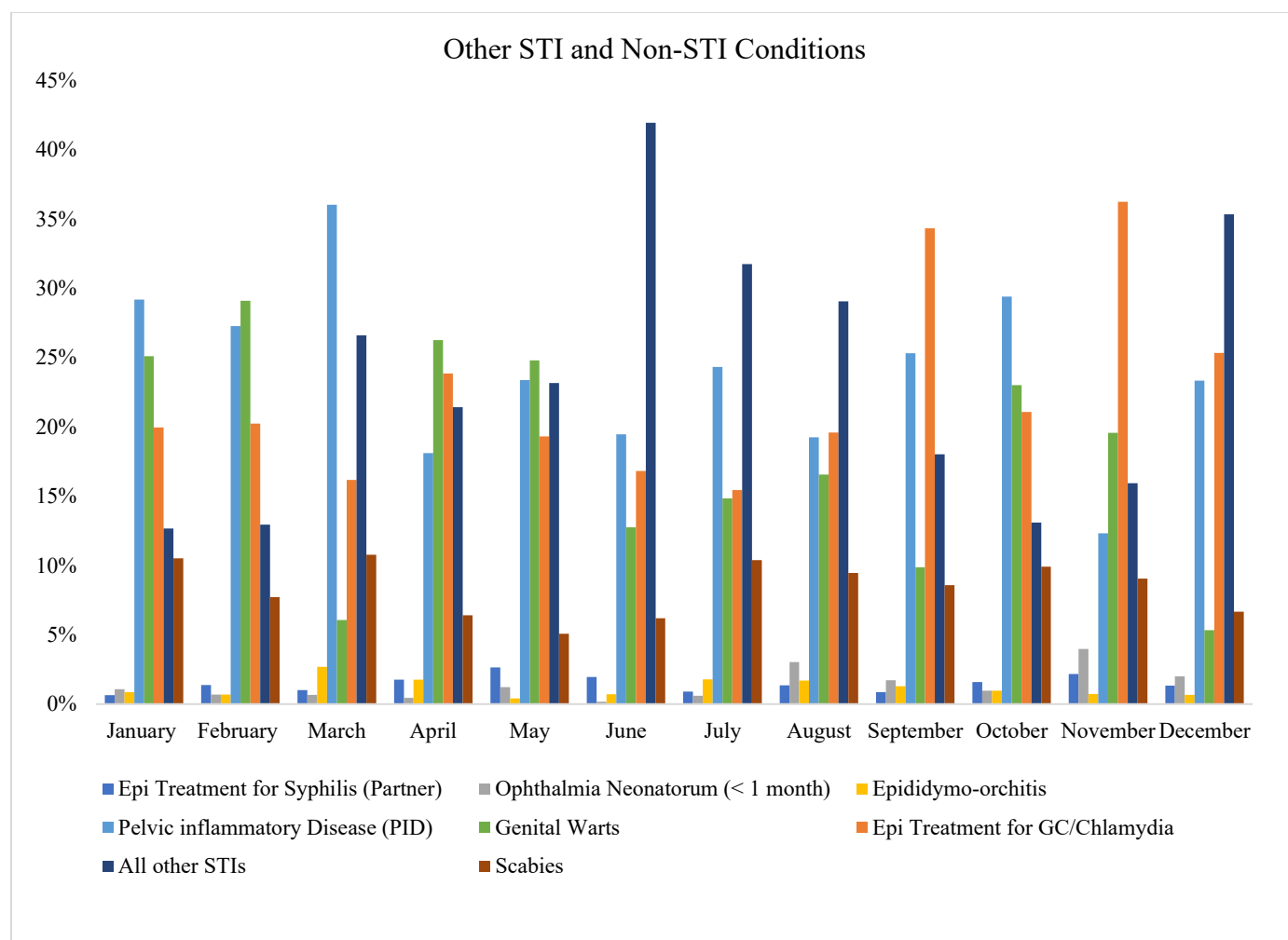


Figure 2.6 Other STIs and Non-STI Conditions

STI Clinic Testing and Surveillance

Table 1. 1 STI Clinic Testing Summary (January- December 2025)

Indicators	South-East		Western		Southern		North-East		Total		
	M	F	M	F	M	F	M	F	M	F	Total
ST Clinic Attendees	3,990	12,238	1,826	6,122	1,009	3,184	976	4,386	7,801 (23.1%)	25,930 (76.9%)	33,731
Tested for HIV	3,015	6,141	922	2,306	799	2,115	695	1,990	5,431 (69.6%)	12,552 (48.4%)	17,983 (53.3%)
Positivity Yield (HIV)	39 (1.3%)	74 (1.2%)	27 (2.9%)	32 (1.4%)	41 (5.1%)	56 (2.6%)	5 (0.7%)	4 (0.2%)	112 (2.1%)	166 (1.3%)	278/ 17,983 (1.5%)
Tested for Syphilis	2,981	6,086	838	2,077	744	2,025	687	1,923	5,250 (67.3%)	12,111 (46.7%)	17,361 (51.5%)
Positivity Yield (Syphilis)	77 (2.6%)	52 (0.9%)	23 (2.7%)	13 (0.6%)	29 (3.9%)	38 (1.9%)	16 (2.3%)	15 (0.8%)	145 (2.8%)	118 (1.0%)	263/ 17,361 (1.5%)

During the reporting period, a total of 33,731 individuals attended STI clinics nationally. Of these, 7,801 were males and 25,930 were females. HIV testing was conducted for 17,983 attendees, representing an overall testing coverage of 53.3%. Males had a higher testing uptake, with 69.6% (5,431) of attendees tested for HIV, compared to 48.4% (12,552) of females. The HIV positivity yield was also higher among males at 2.1% (110 cases), compared to 1.3% (166 cases) among females. Overall, the national HIV positivity yield stood at 1.5% (278 positive cases).

For syphilis testing, 17,361 attendees were tested, representing 51.5% of the total. Similar to HIV testing, males had a higher syphilis testing rate at 67.3% (5,250) while only 46.7% (12,111) of females were tested of the total women (25,930) who attended clinic. The syphilis positivity yield was also significantly higher among males at 2.8% (145 cases) compared to 1.0% (118 cases) in females. This resulted in an overall syphilis positivity rate of 1.5% (263 cases). Notably, the Southern Region recorded comparatively higher positivity yields for both HIV and syphilis during the reporting period, accounting for 35% (97) of all new HIV cases (41 males, 56 females) and 25% (67) of all new syphilis cases (29 males, 38 females) reported nationally.

Table 1.2 Action Sheet

Challenges Identified	Intervention	Timeline	Responsible Person
Late Submissions	Implement automated reminders and follow-ups through DHIS2 to improve timeliness and completeness of report submissions.	January 2026	Sheldon Whorms
Data Timeliness and Completeness	Monitor and report on how promptly each parish/region enters and completes reports, encouraging accountability and timely submissions.	Ongoing	Shanille Brissett Contact Investigators
Data Quality	Integrate data quality checks at weekly and monthly intervals to identify gaps in compilation of data for reporting.	Ongoing	Contact Investigators
Data Presentation	Data to be presented as the reporting rate as opposed to raw numbers to identify true shift in STI indicators.	2026 and onwards	Shanille Brissett